POLICY BRIEF · HEALTH FINANCING · NIGERIA

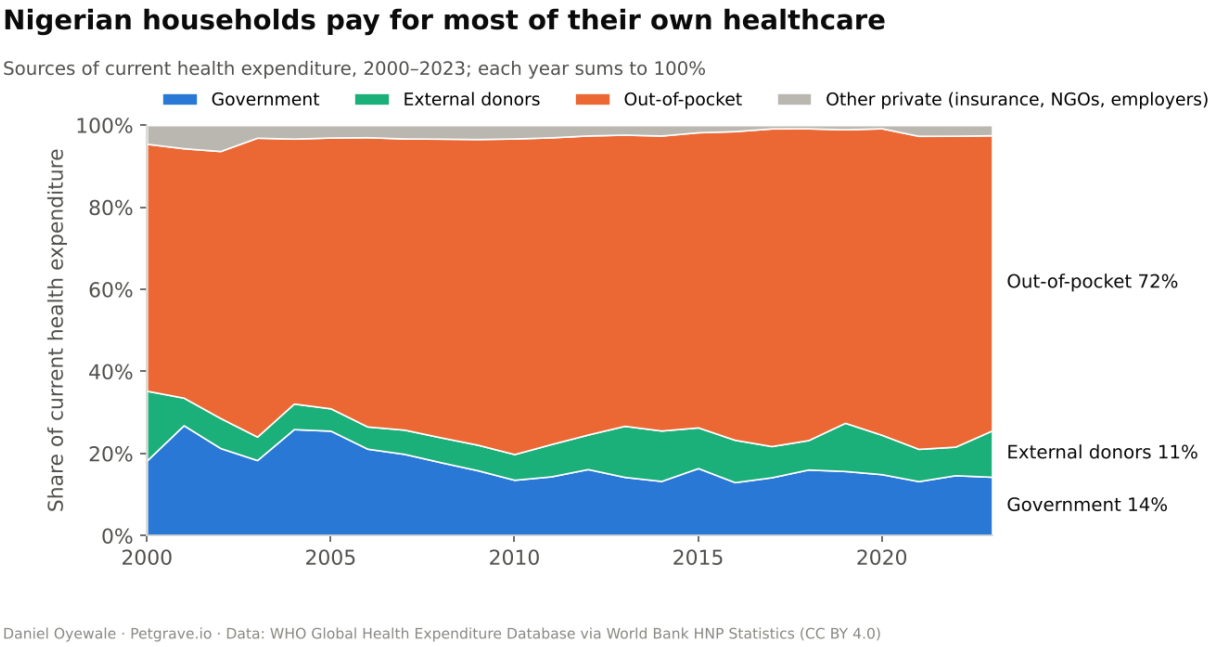
Who pays for healthcare in Nigeria?

Mostly, the patient. In 2023 households paid 72% of all health spending out of pocket, the highest share of 48 countries in sub-Saharan Africa. Public money that replaces those payments, not simply a bigger budget line, is what the evidence links to fewer families in hardship and fewer child deaths.

OUT-OF-POCKET SHARE	HEALTH SHARE OF BUDGET	GOVT SPEND PER PERSON	CATASTROPHIC COSTS	UNDER-5 MORTALITY
72%	**4.3%**	**$10**	**15.8%**	**117**
#1 of 48 in SSA · safe band 15-20%	Abuja target 15%	benchmark about $111	>10% of budget (2018), about 32M people	per 1,000 live births

1. Households carry the system

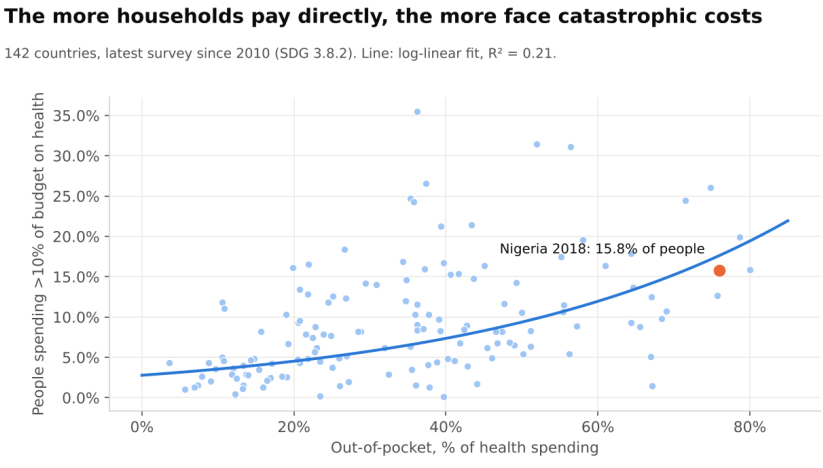
Nigeria spent about $67 per person on health in 2023: $48 from patients' pockets, $10 from government, the rest from donors and other private sources. The out-of-pocket share has stayed between 66% and 77% since 2005, while the government share slipped from 18% to 14%. Health receives 4.3% of government spending, about a third of the 15% that Nigeria committed to in Abuja in 2001.



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2. Paying at the point of care pushes families into hardship

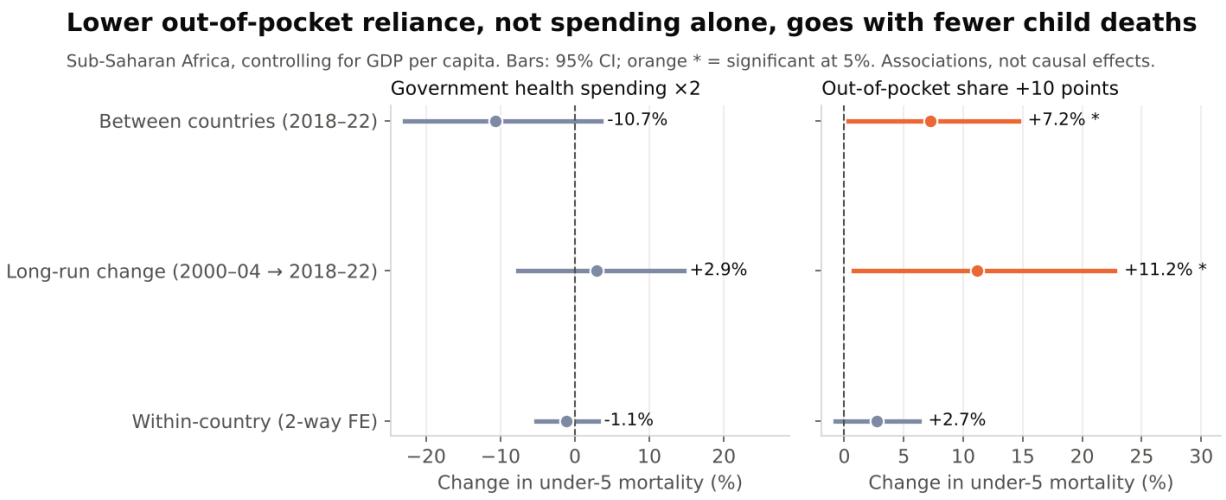
In the 2018 household survey, 15.8% of Nigerians spent more than a tenth of their household budget on health and 4.1% more than a quarter. Across 142 countries, each 10-point fall in the out-of-pocket share goes with about 22% fewer people facing catastrophic costs.



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3. Does more public money buy better health?

Not on its own. In a panel of 46 African countries (2000–2023), year-to-year changes in government health spending are not significantly linked to under-5 mortality (p = 0.60). The out-of-pocket share is: over the long run, countries whose out-of-pocket share fell by 10 points saw under-5 mortality about 11% lower than otherwise (p = 0.035), controlling for income. These are associations, not causal estimates.



4. What would it cost?

Target	Extra per year	% of GDP	Out-of-pocket share	Catastrophic spending	Fewer under-5 deaths a year
Abuja: 15% of government budget	$5.4bn (N8.1tn)	1.5%	72% → 44%	14.3% → 7.3%	62k–223k
5% of GDP	$16.0bn (N24.0tn)	4.4%	72% → 15% (floor)	14.3% → 3.6%	123k–397k *
US$86 per person	$23.0bn (N34.5tn)	6.3%	72% → 15% (floor)	14.3% → 3.6%	123k–397k *

Assumes 60% of new public money replaces patient payments, an out-of-pocket floor of 15%, 2023 prices and N1,500/US$. * Out-of-pocket change larger than any African country achieved in 2000–2022 (max 42 points); mortality effect extrapolated. Catastrophic baseline = 2018 survey value adjusted to the 2023 out-of-pocket share along the cross-country curve.

5. Recommendations

- **Fund the Abuja target in stages.** Reaching 15% of the budget costs about $5.4bn a year (1.5% of GDP, 3.4× the 2025 federal health budget). Phase it in over five years, sharing the cost between federal and state governments.
- **Spend it on replacing fees, not adding line items.** Route new money through the Basic Health Care Provision Fund and state insurance schemes to remove user fees for primary care, maternal and child services first.
- **Pool, then purchase.** Insurance covers about 10% of Nigerians (21.7M). Make enrolment automatic for the poor, subsidised by tax, so prepaid money replaces cash at the clinic.
- **Measure what families pay.** The last catastrophic-spending survey is from 2018. Add health-spending questions to every household survey and publish out-of-pocket share as a budget KPI.

Data: WHO Global Health Expenditure Database via World Bank HNP Statistics; World Bank WDI (CC BY 4.0). Benchmarks: WHO World Health Report 2010; Abuja Declaration 2001; McIntyre et al. 2017. Budget context: 2025 Appropriation Act; Nigeria Governors' Forum; NHIA. Methods, code and an interactive simulator: github.com/Engr-Daniel/health-financing-nigeria.